

EXTRA ENT WITH DR
SHUBHAM

Tracheostomy

M/C indications :

Prolonged
intubation

M/C tube :
used

Portex Cuffed
Tracheostomy tube.

Type

High

1st & 2nd

T. living.



Med

2nd & 3rd

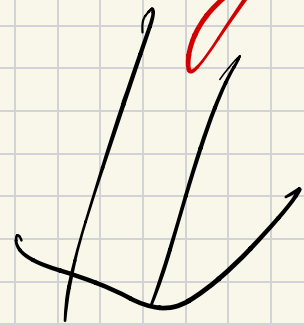
T. living.

(M/C)

Low

3rd & 4th

T. living.



Laryngeal
Cancer.

F/B
aspiration.

Ludwig Angina

→ rapidly spreading
cellulitis of submandibular

Space.

→ Diabetes patients

→ T/t → I & D

+
Mouth Hygiene

+
Control diabetes.

~~Stylogia~~

[Eagle Syndrome]

S.P

Ⓟ

length.

<

2.5 cm.

→

>

4 cm

→

Elongated S.P

↓

G.P nerve

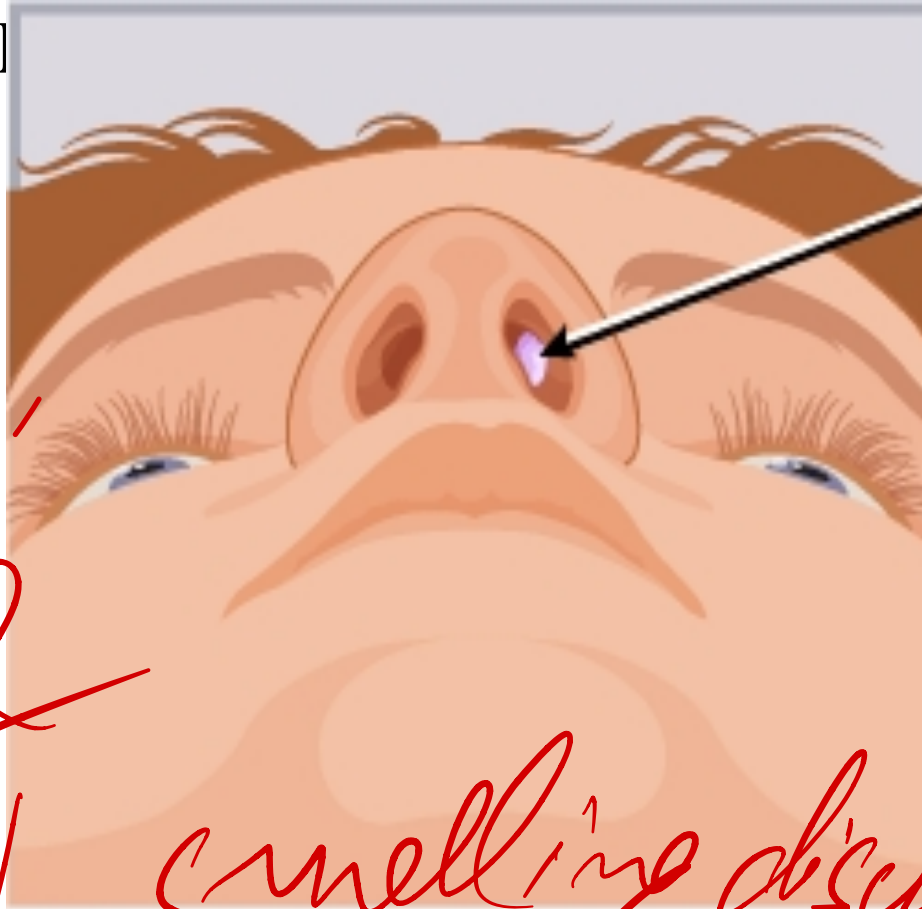
NOSE

Foreign body Nose

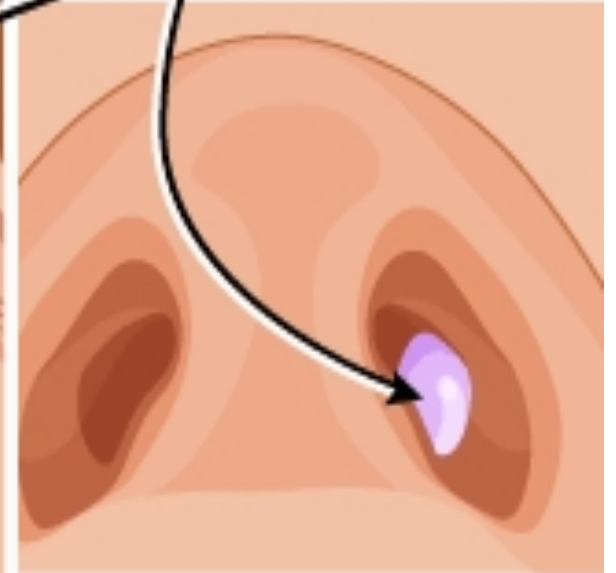
• FOREIGN BODY NOSE

Child

with
V/L nasal
obstruction &
foul



Object



Closer View of Nostrils

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smelling discharge.

BASAL CELL CARCINOMA-RODENT ULCER

→ Occurs in sun exposed areas.



→ Everted margins with ulcer in center.

LARYNGOCOELE

Air inside
sacculus of larynx.

- TRUMPET BLOWES
- GLASS BLOWERS

- EXTERNAL, INTERNAL, COMBINED

- BRYCE SIGN

Hisssing sound
on pulling L. Coele.

I

0 - 50 %

II

51 - 70 %

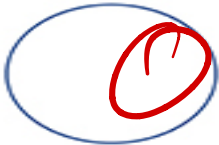
III

71 - 99 %

IV

100 %

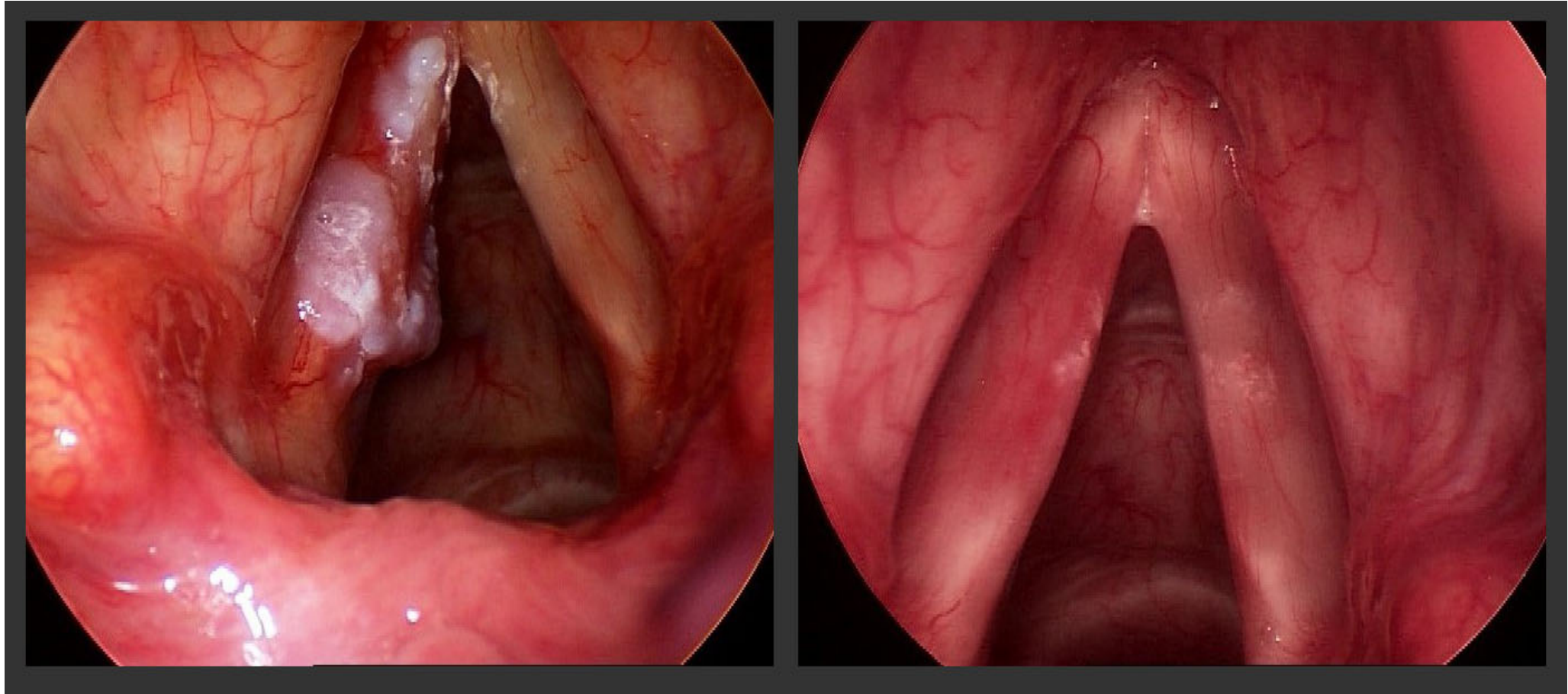
SUBGLOTTIC STENOSIS- COTTON MEYER CLASSIFICATION

Classification	From	To	Endoscopic appearance
Grade I	 No Obstruction	 50% Obstruction	
Grade II	 51%	 70%	
Grade III	 71%	 99%	
Grade IV	 No detectable lumen		

TREATED BY USING ENDOSCOPIC/
LASER DILATATION + MITOMYCIN C

Antibiotic

CANCER LARYNX

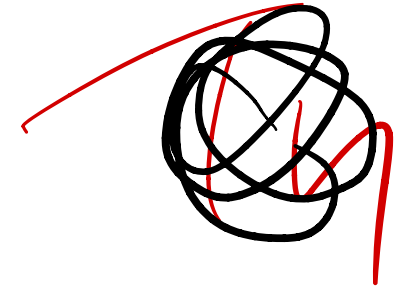


T1- ONLY ONE NAMED STRUCTURE IS INVOLVED

- T1A- ONE VOCAL CORD INVOLVED

- T1B- BOTH THE VOCAL CORD INVOLVED

T2- MORE THAN ONE NAMED
STRUCTURES ARE INVOLVED



T3

- VOCAL CORDS ARE FIXED/ IMMOBILE

• OR

- THERE IS INVASION OF PRE EPIGLOTTIC SPACE OR PARAGLOTTIC SPACE

T4

- INVASION OF THE THYROID CARTILAGE

- OR

- EXTRALARYNGEAL EXTENSION / NECK STRUCTURES

- Eg. THYROID EXTENSION

TREATMENT

- T1, T2

- RADIOTHERAPY

- T3, T4

- **TOTAL LARYNGECTOMY**

- **+/- NECK DISSECTION**

- **FOLLOWED BY
RADIOTHERAPY**

2nd ⇒ Chemoradiation.

- **IF THESE ARE NOT IN
OPTION THEN 2ND BEST**

Q. boy. Male.

Amyotonia only.

a) Vertical P. Large.

b) Total

~~c) Horizontal P. Tdyc + Radiotherapy~~

~~fb chemo~~

d) Radiotherapy

ISSHIKI CLASSIFICATION OF THYROPLASTY

I MEDIALISATION - DONE IN ADDUCTOR PALSY

II LATERALISATION - DONE IN ABDUCTOR PALSY

III SHORTENING/ LOOSENING - DONE IN
PUBERPHONIA

IV TIGHTENING LENGTHENING - DONE IN
ANDROPHONIA

I

M → Medialisation ⇒ S.L.N Palsy.

II

L → Lateralisation ⇒ R.L.N Palsy.

III

S → Shortening ⇒ Hyperphonia.

IV

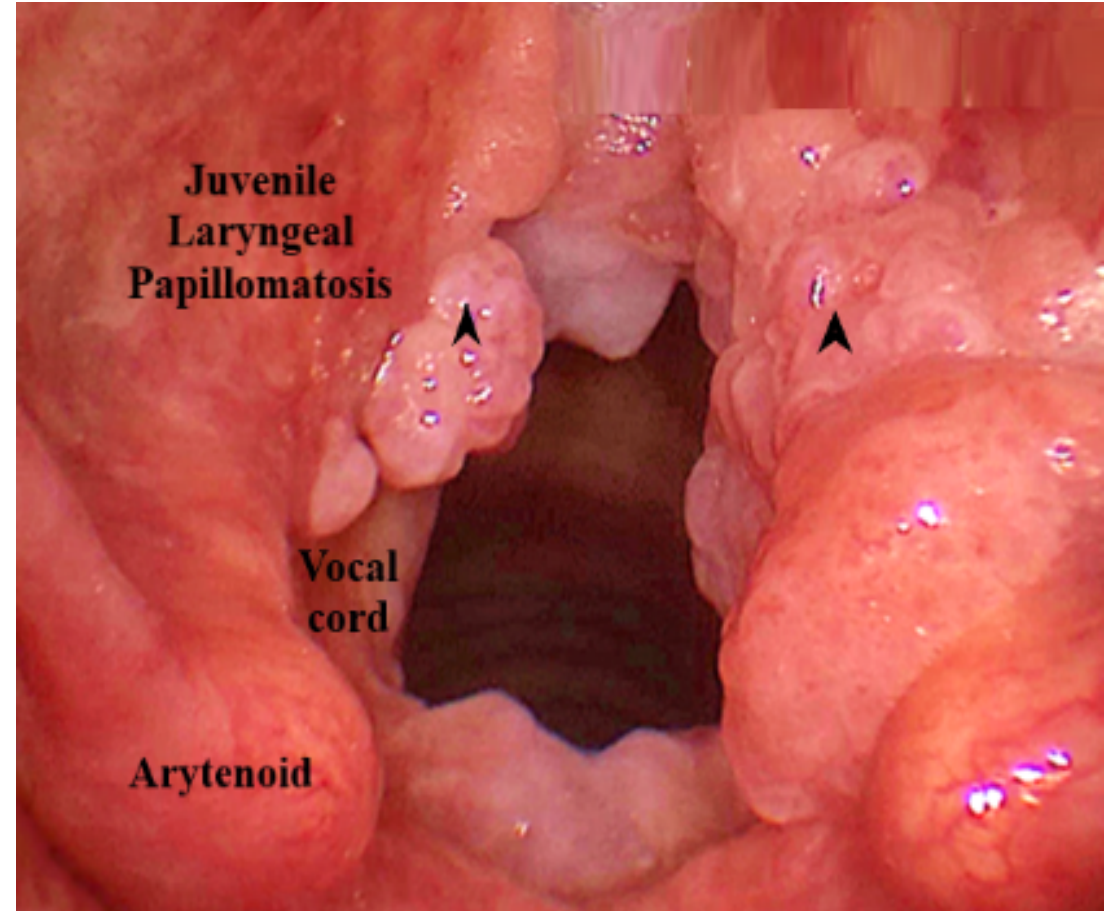
lengthening ⇒ Androphonia.

JUVENILE PAPILLOMA OF LARYNX

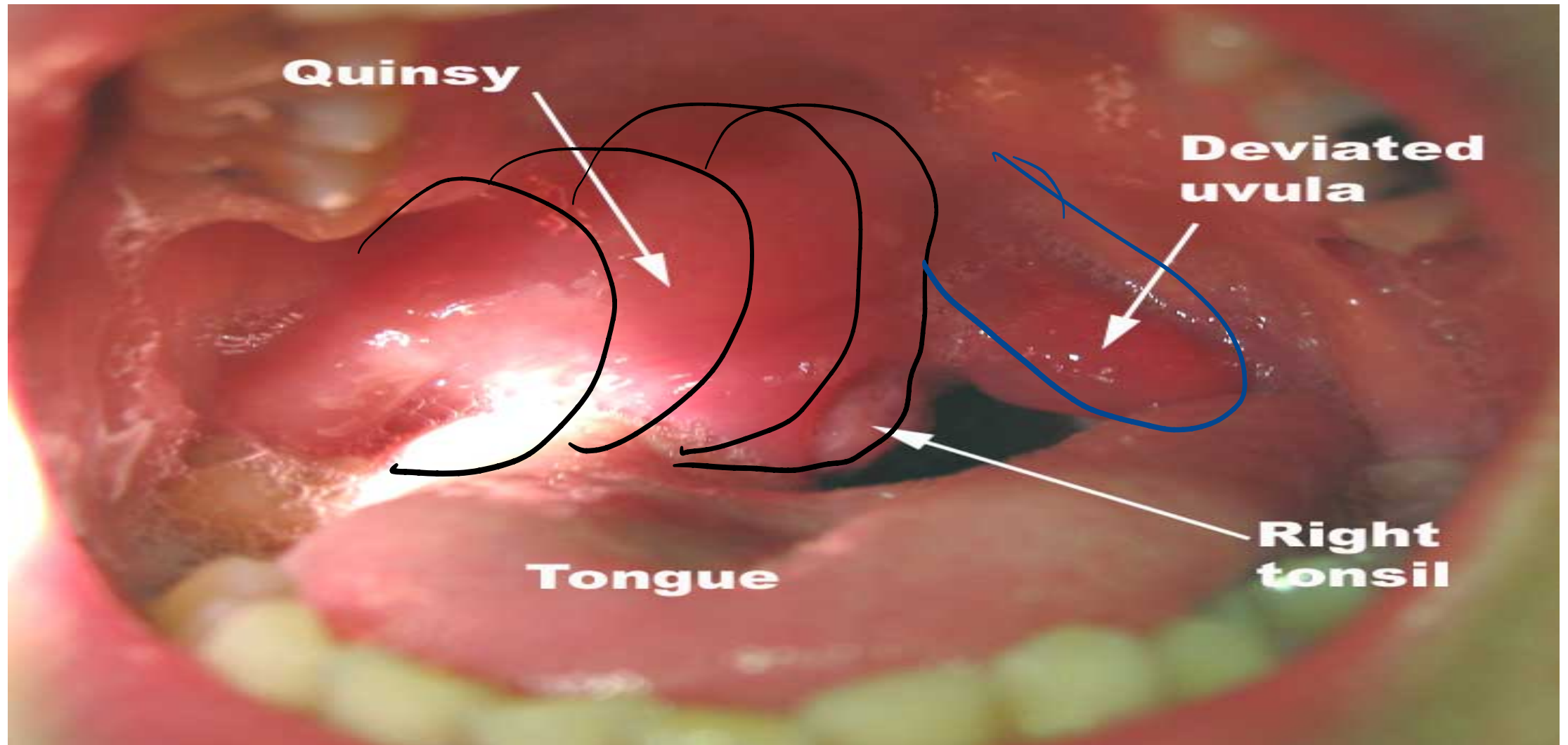
1. • HPV 6, 11
2. • CO₂ LASER CAN BE USED BUT DUE TO EVAPORATION, RECURRENCE IS THERE

• INTRALESIONAL
CIDOFOVIR INJECTION
USED.

Recent
advance
question.



QUINSY - PERITONSILLAR ABSCESS



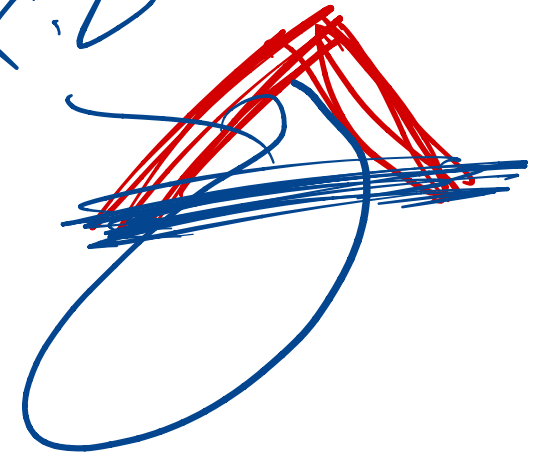
~~T/H → I & D~~

K.D

CLINICAL FEATURES

- HOT POTATO VOICE = PLUMMY VOICE
- TRISMUS - DUE TO SPASM OF MEDICAL PTERYGOID MUSCLE

I.C.



Cont - by - dr. shubham.